



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : صباح احمد معوض المليجي
المحافظة : الجيزة
الحى / المدينة : امبابة
رقم البطاقة : 26705251700828 رقم التليفون : 01068075577
العنوان : -
العمر (تاريخ الميلاد) : 25-05-1967 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 26-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام اشعه مقطعيه علي الصدر بالصبغه [10851] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: صباح احمد معوض المليح National ID: 26705251700828	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Contrast-enhanced chest CT

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
بطاقة تحقيق الشخصية

صباح
احمد معوض الملبجي

٦٨ ش النمر - امتداد ش الوحدة - المنيرة
امبابة - الجيزة

١٩٩٧/٠٥/٢٥

٢٠٦٧ ٠٥ ٢٥ ١٧ ٠٠٨ ٢٨

١٠٢٥١٧١٨٦

DMG

اجبات وفحوصات
شركة مصر
سونار طنز وحوش
سونار شري

٢٠٢٠/٠٩ ٢٦٧.٥٢٥١٧.٠٠٨٢٨

حاصله على دبلوم صناعي

أنثى مسلمة متزوجة

على سعيد محمد على

البطاقة سارية حتى ٢٠٢٧/٠٩/١٦

١

MDT FORM

30 / 12 / 2025 G

Name	صباح احمد معوض	MRN	صباح احمد معوض المراجعي DOB: 1967-05-25 Female MRN: 1087/2025
Age	58	ID	
Menopausal status	Post menopausal	Phone	
NRS SCORE		Marital status	
BMI		Family history	EU.050111.111
CO morbidity	-ve	REASON OF MDT	Singof

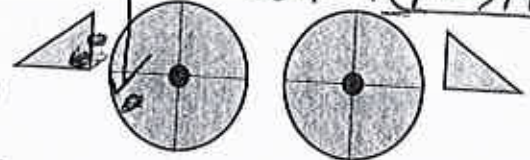
Clinical Data

BX:

ER:	PR:	HER 2:	Ki67:
8/8	8/8	ve	2+

R

Focal skin thickening
no clip -> (Pine) NACT



Sonomammography:

Locally Rt breast & ipsilateral
axilla LUS

Mets/Workup:

Received NACT
X4 AC 12XWK
Taxol

TNM

CT CAP => few bilateral
pulmonary nodules
(millimetric)
Bone scan => Free

Recommendations:

BSM -> (R)

for surgical consult

30 / 12 / 11

MDT OUTCOME & PLAN

for Rt MRM after revision
- still Focal skin thickening
- still no clips
- still noted

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
[Signature]	[Signature]	[Signature]			



Patient Name : الأستاذ / صباح احمد معوض المليجي	Request No. : 13006468
Patient No. : 26705251700828	Request Date : 23/01/2025
Path No. : 323/25	Verification Date : 09/02/2025
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 57 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Right LOQ lesion.

Nature of Specimen: Tru cut biopsy collected at 23/1/2025 at 10.40 am.

Gross Examination:

- Multiple grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Duct carcinoma insitu: No DCIS.
- * Perineural invasion: No.
- * Angioinvasion: No.
- * IPT (Positive control showed appropriate reactivity):
 - ER: Strong nuclear reaction in 90% of tumor cells (score 8 /8).
 - PR: Strong nuclear reaction in 60% of tumor cells (score 7 /8).
 - Her2: No membrane reaction in the tumor cells (score 0).
 - Ki67: 21% nuclear reaction of tumor cells.

Diagnosis:

Right LOQ breast swelling, Slide revision:

Invasive duct carcinoma Grade II.

IPT :

- ER: Positive (score 8 /8).
- PR: Positive (score 7 /8).

Verified by :

DR.Menna Nabil

Printing Date : 22/02/2025 09:49:02

Page 1 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



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Age : 57 Y	Department : Out Patient
Referreing Doc :	Registered by : .marwa mohamed youss

Pathology Report

- Her2: Negative (score 0).
- Ki67: Positive in 21% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

Verified by :
DR.Menna Nabil

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Mrs. Sabah Ahmed Moawad

Date: 22/1/2025

Breast Mammography and Ultrasonography

➤ Complain: right sided palpable mass.

- Right breast shows LOQ spiculated mass lesion with corresponding hypoechoic irregular mass seen by US, showing internal vascularity by Doppler activity with surrounding small satellites as well as desmoplastic reaction, the whole area of concern measures about 5x4 cm.
- Left breast shows normal glandular pattern, with no definite solid or cystic lesions.
- Normal appearance of nipple, areola & skin.
- Right axillary suspicious LN with thickened cortex measuring 13mm.

Conclusions

- Right breast suspicious mass as described above with ipsilateral suspicious LN. BIRADS 4 for biopsy.

Dr. Sabar Monsef



Patient Name: Sabah Ahmed Moawwad	Date: 04/05/2025
Exam: C.T CHEST WITH CONTRAST	Age: 58 Year(s)
Medical ID: 1087/2025	Pat. ID: 99298

**POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

A case with history of right breast cancer.

1. Chest:

- Irregular soft tissue mass lesion seen at right breast (rest of the breast is seen out of field of view).
- Enlarged right axillary LNs (level I&II) , largest seen at level I measuring about 3.2x2.9cm.
- Few bilateral millimetric pulmonary nodules are seen.
- Prominent subcentimetric mediastinal lymph nodes (perivascular and para tracheal).
- No evidence of pleural effusion or masses.
- Cardiomegaly is seen.
- No pericardial effusion detected.

2. Abdomen and pelvis:

- The liver is of average size with homogenous low CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- The spleen is of average size with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No pelvi-abdominal collections.
- Multiple pelvic phleboliths are noted.

3. skeleton:

- No lytic or sclerotic bone lesions.
- Multilevel spondylo-degenerative changes seen with first L4 spondylolithesis

OPINION:

**Few bilateral millimetric pulmonary nodules for follow up
No CT evidence of hepatic or osseous deposits.**

Dr.Mona Ahmed

Prof. Menna Tallah Hanafy, M.D.

ECHO REPORT

Name: *محمد محمد*

Date: 17 / 6 / 2025

EF	54 %	55-75%	PWT	0.7	0.6-1.1cm
LVEDD	5.8	3.5-5.8cm	AO.R	2.7	2-3.7cm
LVESD		2-4cm	LA	4.2	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	0.8	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions *Borderline L.V int dimension*

LV wall thickness *normal*

LV diastolic function *D.D grade I*

SWMA *Mild Global Hypokinesia*

LA and Ao root dimensions *Dilated L.A*

Mitral Valve *Mild TR*

Aortic valve *normal*

Tricuspid valve *Mild TR*

Pulmonary valve *normal*

Right side and systolic function *normal*

Intracardiac masses and effusion *normal*

Borderline L.V systolic function

EF = 54 % Biphasic Simpson's method

So SS I. Eye Balling

Dr / Mina Adel

[Signature]